

mcrc-kras-a59t-pik3ca-peritoneal-pre-hi pec-q8k4

Plain-language summary for patient and caregiver

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What this page is

Libby is a tool that looks for treatments aimed at the specific genetic changes in one person's cancer. It read the details of your case, searched the published studies and open clinical trials, and put together four options that target the particular mutations found in your tumor. This page walks through those four in plain language: what each one is, what the upside might look like, what the risks are, and where the evidence is strong or thin.

A few things to hold onto as you read. Libby only looks at options tied to your tumor's targetable features. The rest of your care — the chemotherapy you're on now, the surgery that's being planned, and the standard drugs that go with all of that — is your oncology team's job, and it isn't on this page. That doesn't make those things less important. It just means they live in a different conversation. And none of what follows is a treatment plan. It's material to bring to your oncologist so you can ask sharper questions.

What we know about your cancer

You have colon cancer that has spread beyond the colon. It reached your liver, where the spots were removed with surgery, and it's now present on the peritoneum — the lining of the belly. You're on your first course of chemotherapy (a combination called FOLFIRI plus bevacizumab), and a specialized operation is being planned: surgery to remove visible disease, followed by heated chemotherapy washed through the abdomen during the same procedure. That operation is usually written as CRS-HIPEC.

Your tumor's genetic testing turned up two changes that most of these options aim at. One is in a gene called KRAS — specifically a form called A59T, which is unusual. Most KRAS-targeting drugs were built for the common forms of the mutation, not this one. The other is in a gene called PIK3CA, a change written as M1043I, which switches on a growth pathway inside the cancer cells. Your tumor also tested as what's called MSS (microsatellite stable) with low TMB and negative PD-L1 — three findings that, taken together, usually mean the standard immunotherapy drugs won't help. One of the options below is an exception to that rule, which is why it's here.

One practical note: some of the genetic results are still being confirmed with a fuller round of testing. That confirmation matters for a couple of the options, and we flag where.

What you told us matters most

You said you lean toward going after the cancer, and you're willing to accept more side effects to do it — you set that dial at about 70 percent toward effectiveness. You didn't rule out any particular side effect ahead of time, and you didn't rule out any type of treatment (pills, infusions, and so on are all on the table). You also said you're open to clinical trials, and you'd rather see specific named trials than vague categories. The options below reflect that: a couple of them are trials.

A first step that shapes everything below

Before leaning hard on some of these options, your team will likely want one more round of tumor testing to nail down the exact genetic results. This isn't a treatment — it's a lab test on tissue that's already been collected, so it carries no risk to you. It matters because two of the options below (the aspirin and the PI3K-pathway trial drug) rest on confirming that the PIK3CA change is really driving the cancer, and the KRAS option depends on pinning

down the exact KRAS result so a trial can decide whether you qualify. It's worth getting that testing moving early so the answers are in hand when decisions come up.

The other thread running through all of this is timing around your surgery. Several of these options interact badly with an operation — some raise bleeding risk, some slow wound healing, some can cause inflammation that would complicate recovery. None of them are things to start the day before surgery. Where timing matters, we say so.

The options

Option 1 — Low-dose aspirin aimed at the PIK3CA change

This is the most solid option on the page, and the simplest: a low daily dose of ordinary aspirin. It's here because of your PIK3CA mutation. A large, well-run randomized trial (called ALASCCA) gave aspirin to colon cancer patients whose tumors carried a PIK3CA-type change, and the group that got aspirin had fewer cancers come back. In numbers you can hold: over three years, about 8 percent of the aspirin patients had a recurrence, versus about 14 percent of those who didn't get it. Two other studies pointed the same direction. So the signal is real and it lines up with your tumor's biology.

The honest limit is this: that trial studied people whose cancer had been fully removed and hadn't spread, not people with active disease in the belly like yours. So the biology travels to your case, but the exact size of the benefit doesn't carry over cleanly — I can't give you a precise number for how much it would help you specifically. The fair way to think about it is as a low-risk extra layer you add on top of your other treatment to lower the odds of the cancer returning, not as a drug that fights the disease that's active now.

The main risk with aspirin is bleeding, and it's modest — in the trial, serious bleeding from the gut happened in about 3 percent of aspirin patients versus about 2 percent without it. The one real catch is timing: aspirin thins the blood, so it has to be paused around your surgery and it stacks with the bevacizumab you're already on. The plan would be to hold it through the operation and add it back afterward, during the recovery-and-maintenance stretch, which is exactly where the trial's evidence lives. It fits what you told us — it's a pill, it's cheap, over the counter, and it doesn't cross any of your lines. The main open question is just the confirmation testing on the PIK3CA result.

Option 2 — An immunotherapy pair (botensilimab plus balstilimab)

This option carries caveats. Here's the tradeoff to weigh.

This is two immunotherapy drugs given together that wake up the immune system against the cancer. It's notable because the standard immunotherapy drugs usually don't work for tumors like yours (the MSS, low-TMB, PD-L1-negative combination normally closes that door). This particular pair is the one exception with a genuine track record in this setting — and, importantly, the strongest signal showed up in exactly your situation: people whose liver spots had been removed and whose disease was mainly in the belly. In that group, roughly 1 in 5 saw their tumors shrink, and the study specifically noted responses in the peritoneum, your main area of disease.

Here's what to weigh against that. The evidence is early — it comes from a small study where everyone got the drugs, with no proper comparison group, so those response numbers are promising but not yet confirmed by the kind of head-to-head trial that settles the question (that larger trial is underway but hasn't reported). And the side effects are real: because these drugs unleash the immune system, it can turn on healthy organs. Serious immune-related problems — most often inflammation of the colon or the liver — happened in roughly a third of

patients. That's a lot, and it matters more for you because immune inflammation overlapping with your planned surgery could delay or complicate recovery. No one died from the treatment in the reported group, and there's a well-established playbook (steroids and other medicines) for managing these reactions.

There's also a timing point that isn't a caveat so much as a fact about when this becomes available: the trials that would enroll you require having tried and progressed through standard chemotherapy first. So this isn't a right-now move — it's an option for later, if your current chemo stops working or after your surgery. If you go this route, it should be at a center that runs these immunotherapy trials regularly and has a standing plan for catching and treating the immune side effects early.

Option 3 — A targeted pill for the PIK3CA pathway (a mutant-selective PI3K-alpha inhibitor)

This option carries caveats. Here's the tradeoff to weigh.

This is a newer class of pill designed to shut down the same growth pathway your PIK3CA mutation switches on — but more precisely than older drugs, hitting the mutated version while mostly sparing normal cells. The appealing part is that one of these trials (a study of a drug called CGT6297) has a group written specifically for colon cancer with a PIK3CA change, which is an unusually good match on paper for your tumor.

Two things pull the certainty down, and they're worth being straight about. First, there's almost no evidence yet for how this class works in colon cancer specifically. The encouraging results so far come from other cancers — mostly breast — and some of them are early conference reports rather than finished, published studies. In colon cancer, the track record is only a couple of responses out of a larger group. Second, and this is the more technical point: your particular PIK3CA change (M1043I) is a less common, "off-hotspot" version. The drugs in this class were built and tested mainly against the more usual PIK3CA changes. When a mutation sits off the well-studied hotspots like this, it's a weaker signal that the drug will help — a less sure bet than if you'd had one of the classic changes. That makes this a more uncertain option than it might first appear, and the confirmation testing could sharpen the picture. There's also a wrinkle from your KRAS mutation: it can partly route around this kind of drug, which is another reason to keep expectations measured and why the drug might work better in combination than alone.

The main side effect of this class is high blood sugar, which needs monitoring and can usually be managed; the newer, more selective versions cause much less of it than the older ones. High blood sugar is also awkward to manage right around surgery, so timing would matter here too. This is a trial-only option, which fits your openness to trials — the practical first step would be checking whether that colon-and-PIK3CA trial group is actually open and taking patients.

Option 4 — Drugs that go after your specific KRAS mutation (pan-RAS / RAS(ON) inhibitors)

This option carries caveats. Here's the tradeoff to weigh.

This is the only option on the page that reaches your KRAS mutation directly, and that's the entire reason it's here. Your KRAS change (A59T) is an unusual one that the well-known KRAS drugs can't touch. A newer group of drugs works differently — they grip the KRAS protein at a spot that doesn't depend on which exact mutation you have — and on that basis they might be able to engage your A59T change. Some of the newest trials are written to accept "any KRAS mutation," which could open a door for you.

Now the honest part, and it's important: no one has ever shown these drugs actually work on your specific A59T mutation. Not in a trial, not in a person. The reason to think they might is a laboratory and structural argument — the way the drug binds suggests it could reach your mutation — but that's a well-reasoned hunch, not proof. The published results for this class come from a different cancer (pancreatic) and from the common KRAS mutations, not yours; the colon cancer numbers that do exist are low and come from a company update rather than a full published study. So the right way to hold this is as a mechanism bet: a scientifically reasonable idea worth exploring through a trial, not a treatment shown to help people with your mutation. It should never be presented to you as more than that.

Because it's a bet rather than a proven option, the single most useful next step is a phone call: someone on your team asking a trial's sponsor whether they'll accept your specific A59T result as qualifying. That's an eligibility question, and it turns on how the sponsor reads their own rules — it's separate from whether the drug works. The side effects of the lead drug in this class are meaningful (rash, mouth sores, diarrhea, at a rate similar to other targeted drugs), and some of the trial versions combine it with other drugs — including one that affects blood-vessel growth and would need to wait until after your surgery has healed. This is a later-line, trial-based path, to line up once your KRAS result is locked in and your surgery is behind you.

Questions to ask your oncologist

- The aspirin option is built on a trial in patients whose cancer had been fully removed, not active disease like mine. Do you think it's still worth adding as a low-risk layer, and when — before or after my surgery?
- If I take aspirin, how will we handle the bleeding risk alongside the bevacizumab I'm on and around the operation?
- What's the timeline for the confirmatory tumor testing, and does it change any of these options depending on what comes back?
- For the immunotherapy pair, what would the plan be for catching and treating immune side effects early, given I have surgery coming up — and does that center have experience running these trials?
- When would the immunotherapy or the trial options realistically become available for me, relative to where I am now with chemotherapy and surgery?
- For the PI3K-pathway trial drug: given my PIK3CA change is an off-hotspot type, how much less certain does that make it, and is there a trial group actually open for colon cancer?
- For the KRAS option, can someone contact a trial's sponsor to ask whether my specific A59T result qualifies me to enroll?
- Is there any of this we should be lining up now versus waiting until after the surgery, so we don't lose time?

Sources

- PubMed references: 23094721, 29401002, 38871975, 38889377, 39476340, 40056080, 40961426, 42090791
- Clinical trial identifiers: NCT04929223, NCT05608044, NCT05768139, NCT06336902, NCT06447662, NCT06751524, NCT06884618, NCT06973564, NCT06993844, NCT07152821, NCT07252479, NCT07383506, NCT07397338